Speaker Runbook: Imaging Center Automation Dinner

> ### 📌 UPDATED 2026-10-22, AFTER THE LUIS REHEARSAL. READ THIS FIRST.
>
> **The dinner:** now **Wednesday 2026-11-04 or Thursday 2026-11-05, after 5 pm** (moved from 10-29; see the 11-01 update below). Rick has court by Zoom (final) on Wednesday 10-28. Attendees: **Luis, Fernando Escarzaga (spelling confirmed), and Jesse Martinez** (an engineer; quiet, analytic, humble). A quiet place and a conversation, not a pitch to an audience.
>
> **Deck is now 30 slides (27 no-numbers).** New: slide 6 (what an AI agent is), slide 11 (one order through prior auth, with the working demo, live at https://rfinlay.github.io/imaging-priorauth-demo/), slide 26 (what the dates depend on). The appendix is 28 (divider), 29 (design, four milestone payments) and 30 (build, PROPOSED figures, red tag until Rick confirms). Every slide number in this runbook has been updated.
>
> **Timeline:** opening targeted around **June 2027, April at the earliest.** The **build starts ~March 2027** so prior authorization works before opening. Say "your new center" only: **no building, property, address, or prior business at the site, ever.**
>
> **Pricing:** design $30,000 in four payments ($6k on signing, $6k at vendor evaluation + demo, gate, $9k, $9k). Build is separate: prior authorization **$40,000 in five $8,000 milestones (PROPOSED)**. Optional support **$1,500/month, two-year term, or per patient (PROPOSED)**. Fernando found $30k in two phases large; their lawyer staged 4 to 5 fixed payments. Luis asked how you make money: answer under "How much?".
>
> **Fernando, per Luis:** straightforward, tough, tight with money; distrustful after vendors over-promised; visual ("When can I see it working?"). He will bring his own ideas. Give him the floor.
>
> **UPDATED 2026-11-01, after the 10-31 call with Luis.** The dinner is now **Wednesday 11-04 or Thursday 11-05, after 5 pm** (Luis picks you up; day not yet locked). **Slide 19 hands off to a LIVE RadAssist Spine MRI demo**, about three minutes, on a synthetic lumbar case. Spine shipped 10-31 from Luis's template, live and tested in a day, every rule his. Pre-dinner checklist and the case card are under "The live RadAssist demo" below; the talk track is in the SPEAKING NOTES (slide 19) and DEMO-SCRIPT Part A.

Date: Wednesday 2026-11-04 or Thursday 2026-11-05, after 5 pm (Luis to lock the day), El Paso, Texas, private dinner **Audience:** Dr. Luis Ramos (champion, existing RadAssist user), **Jesse Martinez** (engineer; quiet, analytic), and **Fernando Escarzaga** (founded **Southwest X-Ray**, five El Paso centers, sold to Akumin 2019-08-16; the skeptic and the signer). **CONFIRMED by Rick 2026-10-01: Fernando Escarzaga is the partner, and Southwest X-Ray is his former company.** No longer an open question. He is an **RDMS sonographer and RPA**, clinical by origin, born 1956. **He did NOT found Desert Imaging** (that was Leroy Candelaria, a radiopharmacist) - never say he did. Co-owner **Veronica Escarzaga** was a named seller; do not assume he is the only decision-maker. Full profile: `~/OneDrive/Desktop/Luis-Partner-Profile-Sherlock-2026-10-01.md` . **Speaking time:** about 43 minutes **Deck:** `imaging-center-automation-deck-2026-10-20.html` (30 slides: 27 of pitch and a three-slide appendix after the Thank You; press `N` for notes, `Esc` for overview; open with `?nonumbers` for the 27-slide variant with no price) **Leave-behind:** `imaging-center-automation-leavebehind-2026-10-20.html` (two pages; print one copy per person, three copies)

Step 0: the confirmation text to Luis, send 2026-10-06 or 10-07

Approved 2026-10-01. Paste as-is. Three asks is the ceiling; a fourth reads like a questionnaire.

```
Luis — I did the brainstorming I promised. There's more here
than I expected, in a good way.

Let's do the week of October 20. I can work around whatever
evening suits you and your partner. Give me the day and I'll
be there.

I'll bring 40 minutes on four things he asked about:
scheduling and patient intake, answering patient calls,
medical records requests, and the throughput reporting an
owner actually wants. Plus one he didn't mention that I think
is the biggest one, which I'll save for the room.

Three things that would help me prepare. Roughly how many
studies a day and which modalities you're planning for year
one. How many people he's thinking of opening with on the
front end. And your partner's name, so I'm not meeting him
cold.

Looking forward to it. Thanks for putting this together.
```

Why each ask is there. *Studies per day and modalities* sizes the operation and is the volume number that has gone unanswered on the RadAssist side for two months; here there is finally a reason for Luis to answer it. *Front-office headcount* is the highest-value question in the text: a man who ran **34 people across five sites** and answers "three" is telling you he is building deliberately lean, which is exactly what you are selling. If he answers "eight," you are selling something different and you want to know before the room. *The name* sits last on purpose, so it reads as a throwaway logistics item rather than making the text about your prep gap.

What is deliberately NOT in this text: anything about his history or the sale. "How long did he stay on after the sale" is genuinely useful, but it is a question about the period he is **currently litigating**. Asking Luis invites Luis to repeat it, and "*Rick was asking about your Akumin years*" is a bad thing for Fernando to hear before you have met. Get it in the room instead, from the man himself, with the "two different ways" question in the ASK section below.

Before you leave the house

1. **DONE 2026-10-06, no action needed: the partner name is in.** Fernando Escarzaga, confirmed. Full name on the deck title slide and leave-behind header; first name in presenter notes and prep docs. Deck + leave-behind PDFs reprinted.**
2. The design fee stays **$30,000**, now in **four milestone payments ($6k, $6k, $9k, $9k), gate after the second.** The **build ($40,000, five $8,000 milestones) and support ($1,500/month) are PROPOSED until Rick confirms;** then remove the red tag on slide 30 and leave-behind page 2 and re-render. Demo live at https://rfinlay.github.io/imaging-priorauth-demo/, linked on slide 11 and leave-behind page 2. Say it once and stop. See the "How much?" objection below for the exact wording.
3. Print the leave-behind. Three copies. Two US Letter pages from Chrome.

4. Charge the laptop. The deck runs with no wifi. RadAssist needs the network only to sign in: do that at home, keep the tab open, and run the pre-dinner checklist in "The live RadAssist demo" below.
5. Open the deck in a browser, press `N` once to confirm notes mode works, press `N` again to hide it. Set the browser to full screen.
6. Do not bring the Sherlock research document. Everything you need from it is in the deck notes and here.
7. **DONE 2026-10-22: Luis moved the opening to ~June 2027, April at the earliest; slides 2 and 9 now say so.**

The live RadAssist demo (slide 19): pre-dinner checklist and the case card

Added 2026-11-01. The demo runs on the production site, radassist.app, on a **synthetic** lumbar case. Nothing you type is a patient. The talk track is under slide 19 in the SPEAKING NOTES and in DEMO-SCRIPT Part A; this section is the preparation and the backup.

Pre-dinner checklist, in order:

- [ ] **Phone hotspot on, laptop joined to it, tested** by loading radassist.app over it. Phone charged, cable in the bag.
- [ ] **Signed in ahead of time, two-factor done.** At home, open https://radassist.app/spine, enter your password and the code from your authenticator app. Leave the tab open. **Do not reload it at the table**; the session is what needs the network, not the report. The report is assembled in the browser.
- [ ] **Spine MRI tab showing a clean study:** Region Lumbosacral, Without contrast, 1.5 Tesla, Comparison None. If the page offers a saved draft, click **New study**.
- [ ] **A plain-text window open** to paste into: TextEdit in plain-text mode (Format, Make Plain Text) or Notes.
- [ ] **The case card below, printed.** One sheet, next to the trackpad.
- [ ] **Backups ready:** the pitch folder's `demo-assets/` holds eight screenshots and a 27-second recording (`spine-demo-recording.mp4`) of this exact case, captured 2026-11-01 from the same build that is live. Open the folder once so it is in Finder's sidebar.
- [ ] **Decide whether to quote Luis** ("it becomes sellable the next day"). Ask him in the car, or let him say it himself.

The case card (synthetic; based on the guide's lumbar test case, simplified and de-identified):

Where	Field	Value
Study	Indications	Other, free text: `Low back pain with right leg radiculopathy`
L4-L5 (the severe level)	Canal stenosis	Severe
	Foraminal stenosis, Side	Severe, Bilateral
	Pfirrmann grade	IV
	Bulge / protrusion / extrusion	Extrusion, AP size 7 mm, Location Central
	Nerve roots	Compression
	Ligamentum flavum	Hypertrophy, 5 mm
	AP diameter of the thecal sac (cm)	0.5
L3-L4	Canal stenosis	Moderate
	Foraminal stenosis, Side	Mild to moderate, Left
	Pfirrmann grade	IV
	Bulge / protrusion / extrusion	Protrusion, 4 mm, Central
	Nerve roots	Contact
	AP diameter (cm)	0.7
L5-S1	Foraminal stenosis, Side	Moderate, Right
	Pfirrmann grade	III
	Bulge / protrusion / extrusion	Bulge, 3 mm
	Nerve roots, Location	Contact, Foraminal
	AP diameter (cm)	1.1
Summaries	Conus	L1
	Discs (summary)	Moderate to severe
	Bones	Endplate osteophytes Moderate; Modic Type II, levels `L4–L5`
Last	L1-L2 and L2-L3	**Mark the remaining 2 levels reviewed (normal)**, then AP 1.2 at each

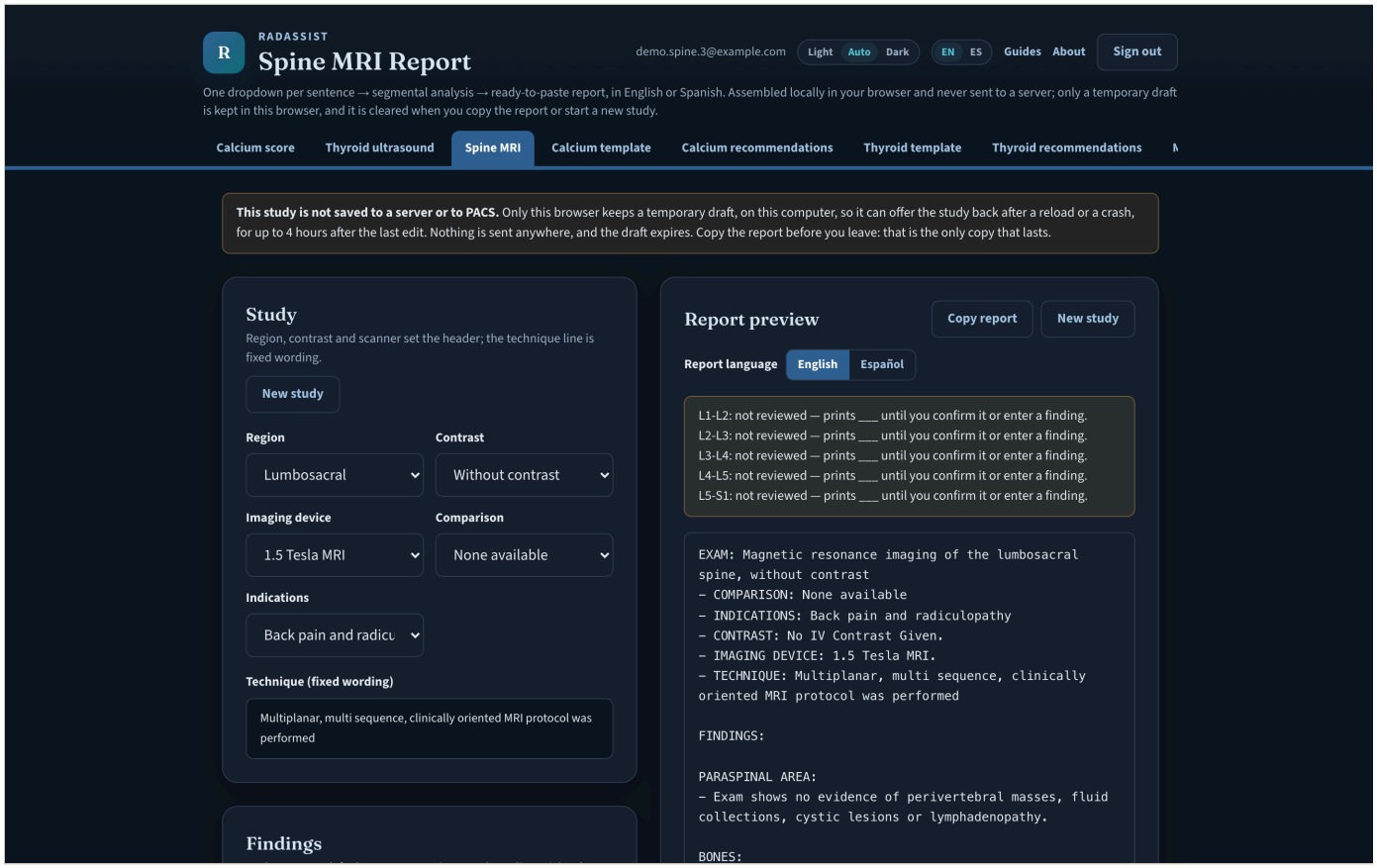
What the impression reads when the case is complete (verified 2026-11-01 on the live build; this is the text Fernando will see):

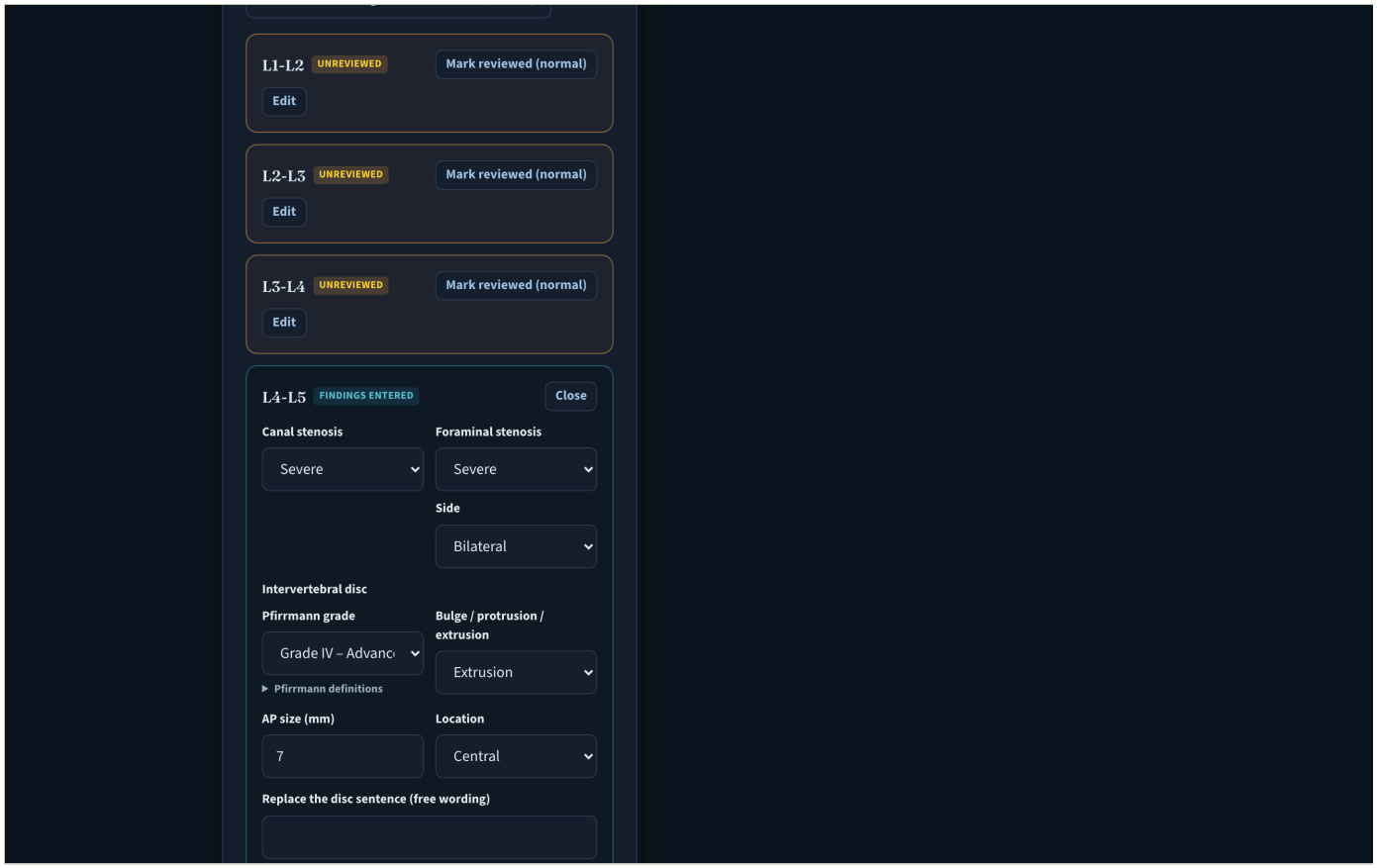
1. No evidence of displaced fractures or dislocations.
2. Moderate to severe degenerative discopathy and spondyloarthropathy, predominantly at the L3-L4, L4-L5 and L5-S1 levels, as described in detail in the body of the report.
3. Severe stenosis of the L4-L5 vertebral canal, the thecal sac and traversing nerve roots are compressed, the residual AP diameter of the vertebral canal is 5 mm.

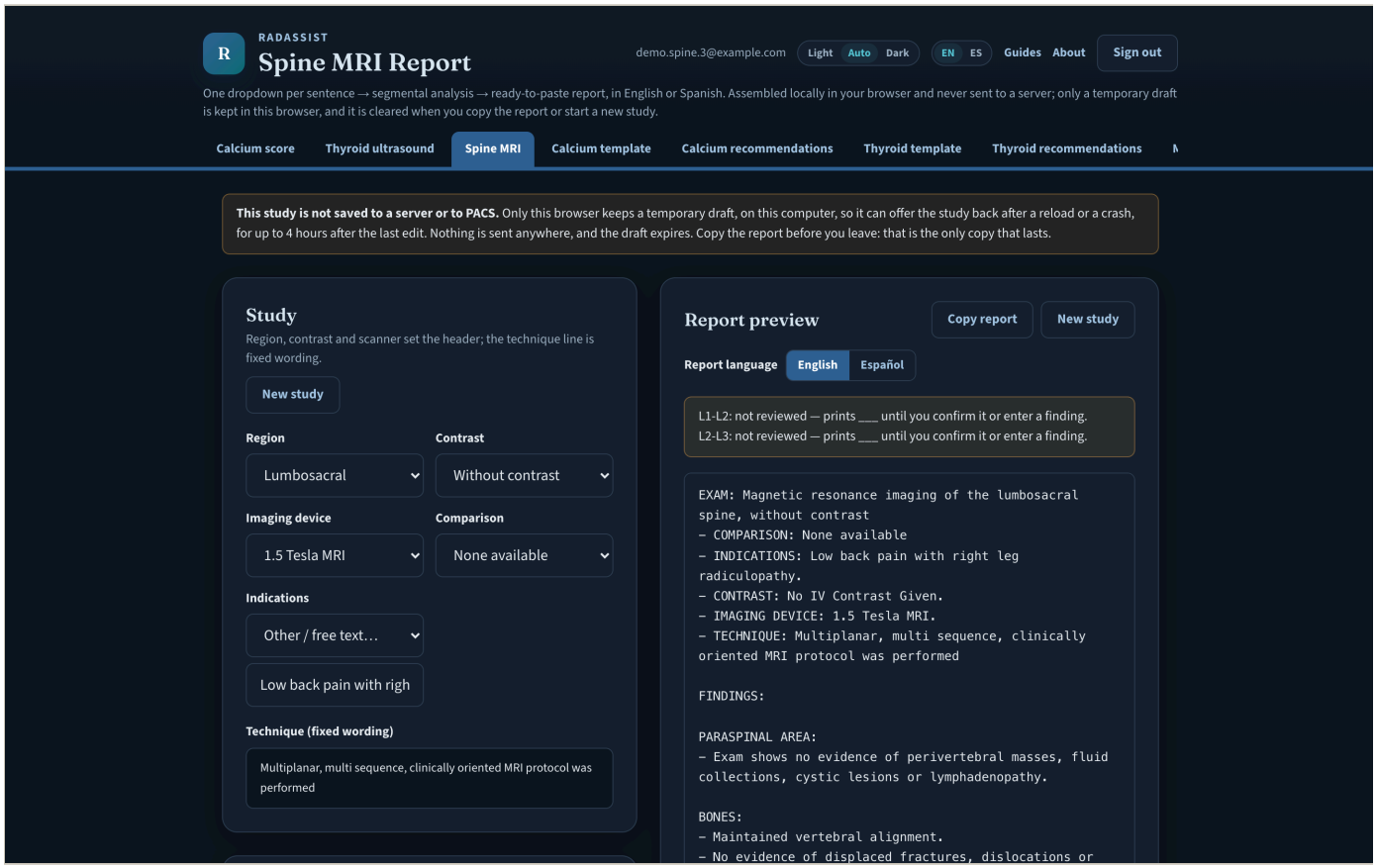
4. Moderate stenosis of the L3-L4 vertebral canal, the thecal sac and traversing nerve roots are contacted, the residual AP diameter of the vertebral canal is 7 mm.
5. Severe stenosis of the bilateral L4-L5 neural foramina, the corresponding exiting nerve roots are compressed.
6. Moderate stenosis of the right L5-S1 neural foramina, the corresponding exiting nerve roots are contacted.
7. Mild to moderate stenosis of the left L3-L4 neural foramina, the corresponding exiting nerve roots are contacted.

Before L1-L2 and L2-L3 are reviewed, an eighth line reads "**Segmental analysis incomplete: 2 disc levels not yet reviewed.**" and those two levels print as blank lines in the body. That is the safety beat of the demo.

Backup screenshots (`demo-assets/` , in the order you would walk them if the tab is gone):




L1-L2 UNREVIEWED
Mark reviewed (normal)
Edit
L2-L3 UNREVIEWED
Mark reviewed (normal)
Edit
L3-L4 UNREVIEWED
Mark reviewed (normal)
Edit
L4-L5 FINDINGS ENTERED
Close
Canal stenosis
Severe
Foraminal stenosis
Severe
Side
Bilateral
Intervertebral disc
Pfirrmann grade
Grade IV – Advanc
Bulge / protrusion / extrusion
Extrusion
Pfirrmann definitions
AP size (mm)
7
Location
Central
Replace the disc sentence (free wording)


RADASSIST
Spine MRI Report
demo.spine.3@example.com
Light Auto Dark
EN ES
Guides About
Sign out
One dropdown per sentence → segmental analysis → ready-to-paste report, in English or Spanish. Assembled locally in your browser and never sent to a server; only a temporary draft is kept in this browser, and it is cleared when you copy the report or start a new study.
Calcium score
Thyroid ultrasound
Spine MRI
Calcium template
Calcium recommendations
Thyroid template
Thyroid recommendations
This study is not saved to a server or to PACS. Only this browser keeps a temporary draft, on this computer, so it can offer the study back after a reload or a crash, for up to 4 hours after the last edit. Nothing is sent anywhere, and the draft expires. Copy the report before you leave: that is the only copy that lasts.
Study
Region, contrast and scanner set the header; the technique line is fixed wording.
New study
Region
Lumbosacral
Contrast
Without contrast
Imaging device
1.5 Tesla MRI
Comparison
None available
Indications
Other / free text…
Low back pain with righ
Technique (fixed wording)
Multiplanar, multi sequence, clinically oriented MRI protocol was performed
Report preview
Copy report
New study
Report language
English
Español
L1-L2: not reviewed — prints ___ until you confirm it or enter a finding.
L2-L3: not reviewed — prints ___ until you confirm it or enter a finding.
EXAM: Magnetic resonance imaging of the lumbosacral spine, without contrast
– COMPARISON: None available
– INDICATIONS: Low back pain with right leg radiculopathy.
– CONTRAST: No IV Contrast Given.
– IMAGING DEVICE: 1.5 Tesla MRI.
– TECHNIQUE: Multiplanar, multi sequence, clinically oriented MRI protocol was performed
FINDINGS:
PARASPINAL AREA:
– Exam shows no evidence of perivertebral masses, fluid collections, cystic lesions or lymphadenopathy.
BONES:
– Maintained vertebral alignment.
– No evidence of displaced fractures, dislocations or

Auto-drafted in Dr. Ramos's order: 1 fractures (the negative sentence, or your finding), 2 post-operative status, then cord compression and pathologic enhancement when present; then discopathy with the most severe level, severe canal stenosis, severe foraminal stenosis, moderate canal, moderate foraminal (mild excluded), other nerve-root findings, listhesis, low conus. Numbered. Edit freely; your wording is never locked.

Impression text

1. No evidence of displaced fractures or dislocations.
2. Moderate to severe degenerative discopathy and spondyloarthropathy, predominantly at the L3-L4, L4-L5 and L5-S1 levels, as described in detail in the body of the report.
3. Severe stenosis of the L4-L5 vertebral canal, the thecal sac and traversing nerve roots are compressed, the residual AP diameter of the vertebral canal is 5 mm.
4. Moderate stenosis of the L3-L4 vertebral canal, the thecal sac and traversing nerve roots are contacted, the residual AP diameter of the vertebral canal is 7 mm.
5. Severe stenosis of the bilateral L4-L5 neural foramina, the corresponding exiting nerve roots are compressed.
6. Moderate stenosis of the right L5-S1 neural foramina, the corresponding exiting nerve roots are contacted.
7. Mild to moderate stenosis of the left L3-L4 neural foramina, the corresponding exiting nerve roots are contacted.
8. Segmental analysis incomplete: 2 disc levels not yet reviewed.

Following your findings — the draft updates as you enter them.

▶ **Thecal sac thresholds**

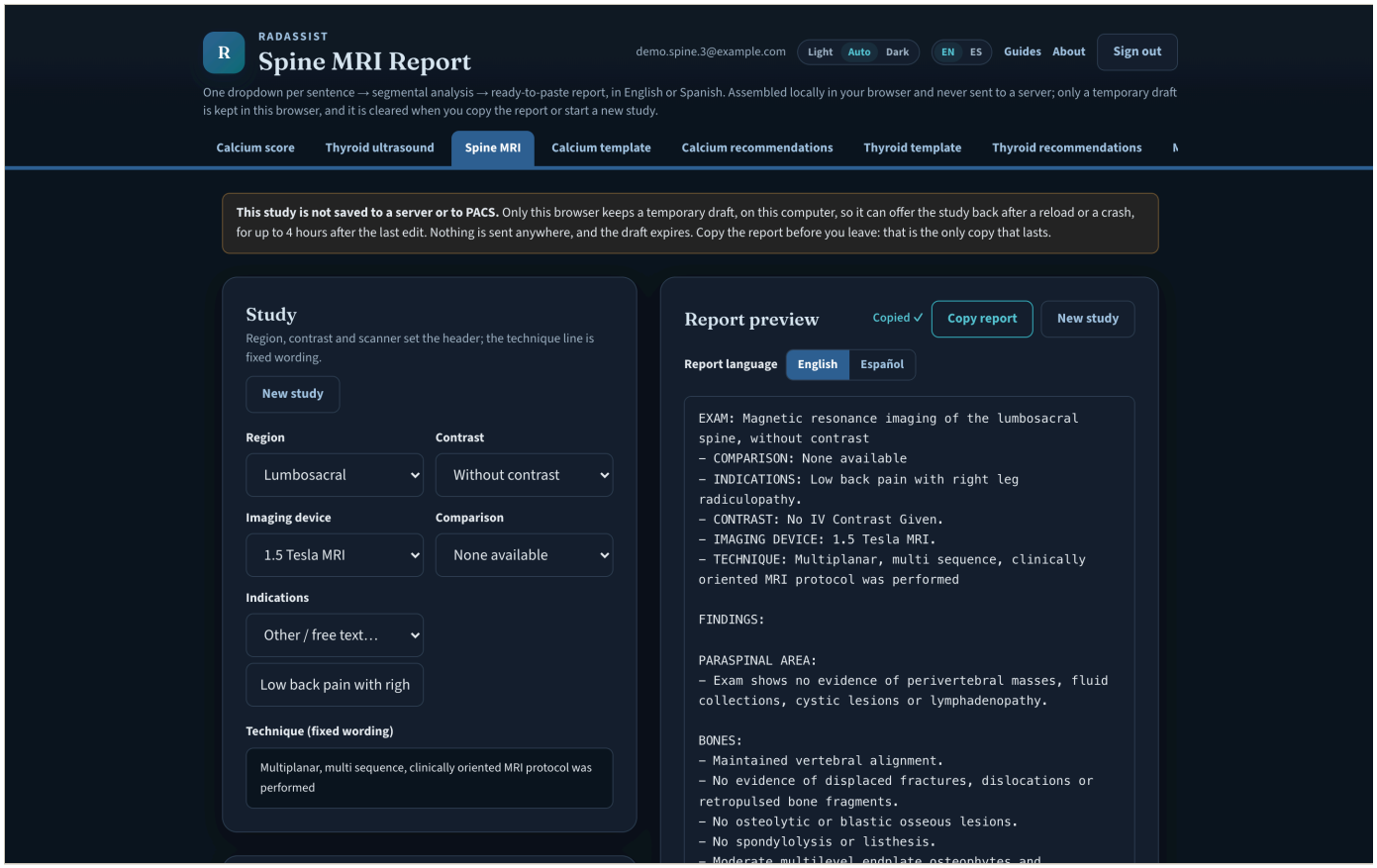
Auto-drafted in Dr. Ramos's order: 1 fractures (the negative sentence, or your finding), 2 post-operative status, then cord compression and pathologic enhancement when present; then discopathy with the most severe level, severe canal stenosis, severe foraminal stenosis, moderate canal, moderate foraminal (mild excluded), other nerve-root findings, listhesis, low conus. Numbered. Edit freely; your wording is never locked.

Impression text

1. No evidence of displaced fractures or dislocations.
2. Moderate to severe degenerative discopathy and spondyloarthropathy, predominantly at the L3-L4, L4-L5 and L5-S1 levels, as described in detail in the body of the report.
3. Severe stenosis of the L4-L5 vertebral canal, the thecal sac and traversing nerve roots are compressed, the residual AP diameter of the vertebral canal is 5 mm.
4. Moderate stenosis of the L3-L4 vertebral canal, the thecal sac and traversing nerve roots are contacted, the residual AP diameter of the vertebral canal is 7 mm.
5. Severe stenosis of the bilateral L4-L5 neural foramina, the corresponding exiting nerve roots are compressed.
6. Moderate stenosis of the right L5-S1 neural foramina, the corresponding exiting nerve roots are contacted.
7. Mild to moderate stenosis of the left L3-L4 neural foramina, the corresponding exiting nerve roots are contacted.

Following your findings — the draft updates as you enter them.

▶ **Thecal sac thresholds**



- Nerve roots: No evident compression or displacement.
- AP diameter of the thecal sac: 1.2 cm
- Ligamentum flavum: normal appearance.

L3–L4: Moderate canal stenosis and mild to moderate left foraminal stenosis.
- Intervertebral disc: Pfirrmann grade IV degenerative changes; disc protrusion measuring 4 mm in AP, central.
- Osseous structures: normal appearance.
- Neural foramina: Mild to moderate left foraminal stenosis.
- Nerve roots: Nerve root contact.
- AP diameter of the thecal sac: 0.7 cm
- Ligamentum flavum: normal appearance.

L4–L5: Severe canal stenosis and severe bilateral foraminal stenosis.
- Intervertebral disc: Pfirrmann grade IV degenerative changes; disc extrusion measuring 7 mm in AP, central.
- Osseous structures: normal appearance.
- Neural foramina: Severe bilateral foraminal stenosis.
- Nerve roots: Nerve root compression.
- AP diameter of the thecal sac: 0.5 cm
- Ligamentum flavum: hypertrophy, measures 5 mm.

L5–S1: No significant canal stenosis and moderate right foraminal stenosis.
- Intervertebral disc: Pfirrmann grade III degenerative changes; disc bulge measuring 3 mm in AP.
- Osseous structures: normal appearance.
- Neural foramina: Moderate right foraminal stenosis.
- Nerve roots: Nerve root contact, foraminal.
- AP diameter of the thecal sac: 1.1 cm
- Ligamentum flavum: normal appearance.

IMPRESSION:
1. No evidence of displaced fractures or dislocations.
2. Moderate to severe degenerative discopathy and spondyloarthropathy, predominantly at the L3–L4, L4–L5 and L5–S1 levels, as described in detail in the body of the report.
3. Severe stenosis of the L4–L5 vertebral canal, the thecal sac and traversing nerve roots are compressed, the residual AP diameter of the vertebral canal is 5 mm.
4. Moderate stenosis of the L3–L4 vertebral canal, the thecal sac and traversing nerve roots are contacted, the residual AP diameter of the vertebral canal is 7 mm.
5. Severe stenosis of the bilateral L4–L5 neural foramina, the corresponding exiting nerve roots are compressed.
6. Moderate stenosis of the right L5–S1 neural foramina, the corresponding exiting nerve roots are contacted.
7. Mild to moderate stenosis of the left L3–L4 neural foramina, the corresponding exiting nerve roots are contacted.

The full-page report preview is `06-full-report-preview.png`, and the top of the pasted text is `08b-pasted-plain-text-top.png`.

The 40 minutes

The deck is 30 slides: 27 of pitch and a three-slide appendix. Re-cut 2026-10-22 after the Luis rehearsal: the agent explainer (6), the prior auth walkthrough with the demo (11), and the assumptions slide (26) are new, and the engagement is now a design slide (29) and a separate build slide (30).

Clock	Section	Slides	What happens
0:00 to 5:00	Open on timing, not technology	1 to 3	Greenfield framing. The four free contract terms.
5:00 to 10:00	The headline number	4 to 5	11.5% appealed, 80.7% overturned. 40% with dedicated prior auth staff.
10:00 to 12:00	What an AI agent is	6	Two kinds, four guardrails, plain words. One line to Jesse.
12:00 to 21:00	The thing he did not ask about	7 to 10	Prior authorization: hours, the federal dates, the 4.94% denial rate.
21:00 to 23:00	One order, start to finish	11	The Tuesday walkthrough. The demo he will click. The Exa answer if it comes.
23:00 to 31:00	The three things he did ask about, corrected	12 to 16	Cancellations, reminders, voice, records.
31:00 to 33:00	Owner reporting and the scorecard	17 to 18	One breath.
33:00 to 37:00	Proof: RadAssist, live, then report assistance	19 to 20	The three-minute Spine MRI demo on the synthetic case. Then Luis describes a Tuesday. The roadmap line. No licensing talk.
37:00 to 39:00	Boundaries	21 to 22	FDA by statute. The compliance envelope.
39:00 to 41:30	**The close**	23 to 27	The bus question. What not to buy from me. If it were my money. What the dates depend on. Thank you. **The no-numbers deck ends here.**
41:30 to 43:00	**Appendix, if the room wants it**	28 to 30	The design, said once. The build, said once (only after Rick confirms the figures). Stop.

Cut guidance, by priority. If you are running long at **27:00**, cut slide 17 (owner reporting); it is the smallest candidate and the scorecard on 16 already names it. If you are still long at **34:00**, shorten the RadAssist demo rather than the close, and let Luis do the talking in whatever time is left; slide 20 can shrink to its heading and the one question. **Never cut slides 23, 24, or 26** (the bus question, what not to buy from me, the engagement, if the number is being shown). Slide 25 is the only close slide that can go, and only if he is already talking, which is a better outcome than the slide.

Protect the close. If you reach 36:00 and are still in the middle, stop wherever you are and jump to slide 23. An unfinished middle costs you nothing. A missing close costs you the engagement.

🔴 LANDMINES: what must never be said to Fernando

Research established these. Each one is a real cost if you step on it.

1. **NEVER mention the Akumin lawsuit.** He is in **active federal litigation against the company that bought his business**: *Escarzaga v. Akumin Imaging Texas, LLC*, W.D. Texas, docket **3:25-cv-00291**, filed 2025-08-01, seeking over **$1,000,000** for age and disability discrimination and retaliation. He stayed on post-sale as Regional Director of Operations, watched Akumin go through Chapter 11, and was terminated without cause or severance. **Not obliquely, not sympathetically, not as "I understand it did not end well."** Revealing you know turns a business meeting into a question of how much you dug. If he raises it: listen, say little, ask nothing, move on.
2. **Never say he founded Desert Imaging.** That was **Leroy Candelaria**, a radiopharmacist. Different man, different company. Telling him otherwise says you confused him with a competitor.
3. **Never say "scale," "platform," "roll-up," or "synergies."** And never "**density in key markets**" - that is near-verbatim from the Akumin press release announcing the purchase of his company. These words describe the period of his life he is currently suing over.
4. **Never say "when you sold to private equity."** He sold to a **publicly traded** company (TSX/NASDAQ: AKU). Private equity arrived later, through bankruptcy.
5. **Never cite Akumin or Stonepeak as a reference customer, a model, or an example of innovation.**
6. **Never frame automation as headcount reduction.** He came up as a technologist and has run departments. He knows the people who answer those phones. If he thinks you are selling layoffs he will disengage politely and you will never know why.
7. **Do not assert his founding year.** Sources say 2003, 2004, and 2006. Let him say it.
8. **Do not mention his age** (born 1956).
9. **Call him Fernando, never Jesus.** Legal first name is Jesus, middle name Fernando; every business-facing source uses Fernando. **Spell it Escarzaga** - Akumin's own release misspelled it "Escargaza" and the BBB says "Escarcega." Both wrong.
10. **Do not treat him as the only decision-maker. Veronica Escarzaga** was a named co-seller and the company held woman-owned designation. Do not discover her at the table.

The reframe that follows from all of this

He is not looking for a platform to scale. He is reclaiming operational control. At 69, having already run a 34-person five-site company and watched the buyer unravel it, he is building lean on purpose.

So **lead with "run this center with a lean front office," never "grow."** He does not want a bigger org chart; he wants the work done without one. That is the strongest honest case for agents that exists, and it is probably why Luis says he has a lot of interest.

And **volunteer your exit terms before he asks.** He has lived the consequences of a signed agreement whose counterparty stopped behaving as expected, so your assurances carry less weight with him than with almost any buyer you will meet. Compensate with verifiable structure, not warmth. Slide 24 is now the most important slide in the deck, not the third most.

The five headline numbers, exact wording, with the caveat you must say

1. 11.5% appealed, 80.7% overturned (slide 4)

Say: "In 2024, Medicare Advantage insurers denied 4.1 million prior authorization requests. Only 11.5% of those denials were appealed. Of the appeals that were filed, 80.7% were partially or fully overturned. Four in five appeals win. Almost nobody files one."

Then say, before anyone asks: "That is Medicare Advantage, all services. It is not an imaging number, because CMS does not collect prior authorization data by type of service. It is the only audited, payer-reported, national figure that exists, and it describes the system your center bills into."

Source: KFF, 2026-01-28, analysis of data insurers report to CMS. 4.1 million denials were 7.7% of 52.8 million determinations.

What it is not: a labor-savings number. It is an abandoned-revenue number. The binding constraint it reveals is staff capacity and follow-through.

2. 40% have staff working exclusively on prior auth; 13 hours per week (slides 5 and 8)

Say: "Physicians and their staff spend an average of 13 hours a week on prior authorization, about 40 requests per physician per week. Forty percent of physicians report having staff who work exclusively on it. You have not hired that person yet."

Then say: "That is the AMA's 2025 survey of a thousand physicians. It is self-reported."

Supporting figures if pressed, same survey: 32% say prior auths are often or always denied; 74% say denials increased over five years; of those who do not appeal, 59% do not expect to win and 52% cite insufficient staff time.

Do not turn this into a headcount-reduction pitch. El Paso wages run roughly 25% below the national average across all occupations (BLS, El Paso MSA mean hourly $24.94 versus $33.54 nationally). A displaced-FTE argument is worth less here than it sounds. Lead with revenue capture and capacity.

3. CMS's 220 million hours (now one line on slide 8, no longer its own slide)

Say: "In its 2024 prior authorization rule, CMS estimated the rule would save providers at least 220 million hours and 16 billion dollars over ten years. They based that on AMA data showing 14 hours per physician per week."

Then say: "That is CMS's projection in the rule. It is not measured savings. Nobody has gone back and counted yet."

Source: CMS-0057-F, 89 FR 8758, published 2024-02-08, Regulatory Impact Analysis.

Note the two AMA figures are different years: 13 hours is the 2025 survey, 14 hours is what CMS cited in the 2024 rule. If someone notices, say exactly that.

4. 5.1% to 3.8% (slide 14)

Say: "A randomized trial at Massachusetts General, about seven thousand MRI patients, added a text reminder on top of the phone call they were already making. No-shows went from 5.1% to 3.8%. It did not improve arrival punctuality."

Then say: "That is 1.3 percentage points absolute. Vendors will tell you that is a 25% reduction. Same study. And only about two thirds of the texting group could actually be reached by text. Your RIS will do this. Let it."

Source: Liu et al., J Am Coll Radiol 2017, PMID 28583321. n=6,989, p=0.02, odds ratio 0.75. Only 67.5% of the texting arm reachable by text.

5. 22.3% cancelled versus 2.0% no-show (slide 13)

Say: "A University of California, Irvine study of about nineteen thousand outpatient radiology exams found 22.3% were cancelled and 2.0% were no-shows. Seventy percent of the cancellations were patient-initiated. Most empty slots are known in advance. That is a backfill problem, not a reminder problem."

Then say: "That is one academic center over 2022 and early 2023. Your ratio will differ. The shape will not."

Source: Aijaz et al., Acad Radiol 2024, PMID 38705764. 19,262 exams, 9,713 patients. 70.19% patient-initiated.

Pocket figure for the scheduling conversation: across 2.9 million outpatient imaging visits over 16 years, booking more than six months out carried an odds ratio of 3.18 for no-show versus booking within a week (Rosenbaum et al., J Am Coll Radiol 2018, PMID 29755001). Shortening time-to-appointment reduces no-shows on its own, and backfill is how you shorten it.

Other numbers on slides, with their framing

Number	Slide	Source	Caveat
4.94% of 17,913 outpatient MRI prior auths denied	10	Harrer et al., J Am Acad Orthop Surg 2025, PMID 39729599	One institution, 2019 to 2022. The point is that 95% get approved, so the labor goes into a process that nearly always says yes.
7 calendar days / 72 hours decision timeframes, 2026-01-01; FHIR Prior Authorization API, 2027-01-01	9	CMS-0057-F rule text	Binds payers, not providers. Covers Medicare Advantage, Medicaid and CHIP, and exchange plans. Does not reach employer-sponsored plans, TRICARE, or VA. The slide now says so; say it out loud before anyone asks.
Almost 63% of medical plans building a FHIR prior auth API	9	2025 CAQH Index (DataSpring), 2026-02-19	Payer self-report.
26.5% of El Paso County residents speak English less than "very well"; 97.8% of them Spanish speakers	14	ACS 2024 5-year, Table B16004	Computed from raw counts. Say 26.5%, not the 66% who speak Spanish at home. The operational number is the limited-English number.
30-day clock, one 30-day extension, 60-day outer limit	15	45 CFR 164.524	Texas has a shorter deadline. Do not quote the Texas day-count.
$6.50 flat fee survived Ciox	15	Ciox Health v. Azar, 435 F. Supp. 3d 30 (D.D.C. 2020)	The court never reached the merits on the fee methods; the option remains.
More than 50 OCR Right of Access enforcement actions	15	OCR	"More than 50" is supportable. Do not quote a settlement figure.
TCPA: one message per day, three per week; $500 per violation, up to $1,500 if willful	20	47 CFR 64.1200(a)(9)(iv); 47 U.S.C. 227	$1,500 is the treble ceiling, not a fixed amount.
Texas SB 1188 US-only storage effective 2026-01-01	20	Bill history at capitol.texas.gov; provision wording from law-firm analyses	Say "as I read it" if pressed on exact wording.

Numbers you must NOT say, even if asked directly

The research could not verify these. If Fernando quotes one of them to you, do not confirm it. Say "I have seen that number and I could not trace it to a source I trust, so I will not use it."

- Any CAQH per-transaction cost (the "$15 manual versus $6 electronic" or "$11 versus $1.50" figures). The chartbook is paywalled and the circulating numbers contradict each other.
- Administrative labor as a percentage of imaging center operating cost. Ask him for his instead.
- Imaging prior authorization denial rates of 25 to 40%. Vendor-blog numbers that contradict the peer-reviewed 4.94% and the KFF 7.7%.
- Any percentage of advanced imaging that requires prior authorization. "Most commercial and Medicare Advantage plans require it for CT, MRI, and PET" is fine as a sentence. A number is not.
- Any vendor's price, for RIS, voice, or prior auth tools. Nobody publishes list prices.
- HL7 interface fee ranges. "Customary, per interface, and negotiable before signature" is the whole statement.
- A Texas HB 300 records deadline day-count or training interval.
- Cost or time per prior authorization, call abandonment rates, days in A/R, phone volume per study, time-to-schedule benchmarks. None of these exist in defensible form.
- Records retrieval cost per request.
- A count of independent imaging centers in the US or Texas.

Added 2026-10-12 from the El Paso payer-mix research. These are false or unverified. Each one has a replacement that is primary-source confirmed.

- **Never "VA requires precertification."** False since 2026-09-08. VA eliminated precertification entirely and took the precertification portal offline. Say instead: "VA dropped precertification in September 2026; the referral and the SEOC govern."
- **Never "TRICARE requires prior auth for MRI, CT, or PET."** For TRICARE Select, For Life, Reserve Select, Retired Reserve, and Young Adult-Select, advanced imaging needs no referral and no pre-authorization; imaging is not on the pre-auth list at all. Only Prime involves a referral. Use the plan-by-plan language in the Fort Bliss objection below.
- **No percentage of the center's imaging VOLUME by payer channel.** The Census measures coverage of residents, not utilization, and imaging skews older and sicker. No defensible conversion exists. Give the coverage shares, then hand the conversion to Fernando: "Volume will skew older than coverage does; you would know the conversion better than I would."
- **Never "185,499 military retirees live in El Paso."** The Texas Comptroller counts retirees with installation access, a catchment wider than the county. Say "with Fort Bliss installation access."
- **No commercial multiple of Medicare for a freestanding imaging center.** The published RAND multiple is for hospital inpatient and outpatient combined; ambulatory surgery centers come in well below it. A freestanding center sits in the lower band, and his actual number is whatever he negotiates. No multiple is on any slide; keep it that way.
- **No VA referral expiry in days.** The regulation caps an episode of care at one calendar year (38 CFR 17.4005). Specific SEOC windows are not primary-source verified. Say: "A VA episode of care is capped at one calendar year by regulation, and the specific SEOC sets the scope. The expiry discipline is what protects the claim."
- **Never "CMS-0057-F requires providers to use the FHIR APIs."** It obligates payers to build and maintain them. Say: "The rule makes the payers build the APIs. Whether you can use them depends on your software vendor, which is exactly the integration question."

- **Never "VA or TRICARE have a FHIR prior-auth API you can integrate with."** Verified false by live probe on 2026-10-12. VA's production FHIR API exposes 19 resources, none of them authorization or referral. Those channels stay referral-and-SEOC workflow, not API.

The CAQH figures that ARE safe: $258 billion in administrative cost avoided in 2024 through electronic transactions, and a $21 billion remaining annual savings opportunity. Both from the 2025 CAQH Index, 2026-02-19. They are not on a slide; use them only if the conversation goes there.

Anticipated objections and your answers

"Doesn't our RIS already do this?"

"For scheduling, reminders, self-scheduling, and intake forms: yes, and it should. I am telling you to buy that and use it. RamSoft's imaging-center product, for one, advertises self-scheduling, automated notifications, and a workflow designer. What I have not seen any RIS in this tier claim is eligibility verification or prior authorization. That gap is where the strongest evidence is, and it is the piece you did not ask about."

Hold the line: do not claim the RIS definitely lacks a prior auth module. Say you have not seen it advertised, and that the specification will verify it against whichever RIS they pick.

"What does Phase 1 actually produce on the vendor side?"

"The interface list. Every interface the automation plan needs, whether each one runs over HL7 version 2 or an API, which direction the data moves, and what the vendor charges for it. In writing, before you sign."

Interface complexity is a Phase 1 deliverable, not an assumption. The contract term (count and fee schedule fixed at signature) protects him from the price. It does not tell him whether he asked for the right interfaces. That is the work.

"Be specific. What does 'API access to your own data' mean?"

Four things, and name them in the contract rather than the word "access": read access to the clinical and scheduling data the center owns; bulk export on demand; the audit and error logs, because you cannot operate what you cannot see fail; and a test or sandbox environment, so changes are not proven in production.

On Rick's own build this answers itself: the center-specific code, its logs, and its database live in a repository and environment under the center's organization.

"What happens when you get hit by a bus?"

Slide 23. Say it in this order:

1. "You own the specification. It is the deliverable of the design engagement whether or not I build anything, and it is written for a competent engineer who has never met me."
2. "You own the code. It lives in a repository under your organization's account. You can revoke my access." Then, raised by you and not found by him in a redline: "One distinction. Everything specific to this center is yours outright, in your repository. Some of what I would build is existing product of mine that I would extend for you. That part you get under a perpetual license you can never lose, with the source in your hands, so you can run it and change it forever, even if I disappear. You are never hostage to me. What you are not buying is the right to sell my product to someone else. If that distinction is a problem, tell me tonight and I will price it differently."
3. "Runbooks, not memory. Every operational procedure is written down: how it runs, how it fails, who to call at each vendor."

4. "No proprietary runtime. Standard interfaces, HL7 v2, DICOM, payer APIs, and a named model vendor under its own business associate agreement. Any integrator can take it over."
5. "The RIS and the voice vendor are contracted in your name. Nothing routes through me that does not have to."

The three tiers, if he asks what exactly is his: the specification, theirs outright. Everything specific to this center (configuration, workflows, integrations, data mappings, reports), theirs outright, in their repository, your access revocable. Existing product of yours that you extend for them (the prior auth engine, the backfill engine, the reporting core), yours, licensed to them perpetually, irrevocably, and transferably, with source. They run it and modify it for their own operations forever. They cannot resell it.

If he asks why the product pieces are not simply his: "It is less expensive for you because I keep the product rights. If you want those too, that is a different number, and I would rather not go there tonight."

If he asks about source code escrow: "Direct ownership is better than escrow. The code for your center is in your repository, so there is nothing to escrow. The product pieces you hold under the license with the source already in your hands, which is the thing escrow is supposed to give you after a fight. You have it on day one, without the fight."

If he asks about a second person: "There is no second person. That is the honest answer, and it is why the engagement is structured so that the specification and the runbooks are the continuity plan, not me."

"How much?"

Say exactly this, once, then stop talking:

> "The design is thirty thousand, fixed fee, fixed scope, paid in four pieces as I deliver. Six when we sign, for the contract language. Six when you have the vendor evaluation and the working demo. Then nine and nine for the written specifications. After the second payment, you decide whether the rest goes."

If the build comes up (figures PROPOSED until Rick confirms):

> "The build was never inside the thirty. It is its own fixed fee, forty thousand for prior authorization, in five payments of eight: running in your RIS vendor's sandbox around March, connected to the payers' test systems, tested by your staff, live before you open, and thirty days of close support after. After that, optional support at fifteen hundred a month, or per patient if you prefer that model."

"**How do you make money?**" (Luis already asked): "Three ways, all in writing: a fixed fee to design it, a fixed fee to build it, and an optional monthly fee to keep it running. I keep the rights to my own product, which is why those numbers stay where they are."

Do not break out hours, name an hourly rate, or justify the number at length. **Never mention that $30,000 is 200 hours at $150.**

If he pushes on price: shorten the term, never cut the rate. Design: "Take the first two milestones and decide the rest when you have seen the work and clicked through the demo." Build: "Pay for the sandbox build, see it run on your RIS, then decide."

Why four payments. Fernando found thirty in two phases large, and their own lawyer staged four or five fixed payments. The rate did not move; the largest single check is nine thousand and each one buys something he can hold.

Recommended ranges, with the reasoning (for Rick): prior auth build **$36k to $48k, $40k proposed**: larger than the design because it runs against two outside calendars (RIS sandbox, payer test environments), carries human approval and an audit trail, a staff test cycle, and thirty days of hypercare. Support **$1,200 to $2,000/month, $1,500 proposed**: they have seen Exa at about $0.90 per patient and Sectra on a multi-year per-patient commitment; flat

monthly in year one because volume is unknown before opening, per patient (around $1.00 per completed study, monthly floor) once real volume exists. Infrastructure (host, database, model usage, backups, monitoring) at cost, estimated in the specification before they commit. Backfill, records, and owner reporting each get their fixed build price in their own specification.

"**Does that change at three centers?**" "No. The design is written once and the four contract terms are negotiated once. What is per-site is the build."

Contract through RGF Global Ventures LLC, never personally. Do not discount because Luis is a family friend; his RadAssist license and retainer are separate.

"We want something of yours live before a specific date" ⚠️ the one you are most likely to get

Updated 2026-10-22: the prior auth build now has a priced, milestone schedule aimed at live before opening, and every date on it is conditional on the slide 26 assumptions. Outside that frame, the 2026-10-10 rule stands: **do not promise anything of yours live by a date.** Not because the work is hard, but because the **infrastructure was never priced.** Anything of Rick's that runs needs a host, a database, an LLM API bill that scales with use, auth, backups, and monitoring. That is a recurring line in *their* operating budget, forever. Promising it live without that number is the same error as quoting a prior auth agent against an unchosen RIS.

The position is neutral, not defensive. Do not explain why nothing of yours is live yet and do not apologize for it. Convert the push into a scoping question:

> "What has to be live the day you open is your vendors' work: the RIS, billing eligibility, the HL7 interfaces. Phase 1 is where I make sure their contracts actually deliver it. My specs come with fixed prices, so when you want a piece built, you'll know what it costs and what it costs to run before you own it. If you want something live before a specific date, tell me which piece and we'll scope it."

The last sentence is the whole move. Not a no, not a yes-with-a-date.

Know the cost of this position before you walk in, so it is not a surprise at the table. You have no day-one deliverable of your own. If Fernando asks "what do I have on opening day," the honest answer is a contracted vendor stack that works plus a set of priced specifications. For a man who has lived through a bad implementation, absence-of-surprises is a real product. But hear that sentence out loud now rather than discovering it mid-dinner.

"Should we hire for prior auth, or will your automation cover it?" ⚠️ the delicate one

Rick's own concern, 2026-10-10, and it outranks every scheduling question in this runbook. Every other question is about Rick's delivery date. This one is about **their irreversible decision.** A center that opens with six front-office FTEs because the automation was not ready has made a hiring commitment that automation later converts into layoffs, in El Paso, in a small market, with Fernando's name on it and Luis practicing locally. Layoffs at a six-month-old imaging center is a story that follows both of them. It cuts the other way too: a center that *under*-hires against a promise that slips has a front office that cannot answer the phone during the exact window that forms its referral reputation. Both failure modes cost more than being wrong about a build date.

Volunteer this. Do not wait to be asked. Saying "I'm not going to tell you to hire thin on my promise" to a man who has run five centers and been through a PE exit is the most credible thing available at that table. Same mechanism as the data-quality caveat: naming the limit of your own pitch is what makes the rest believable. Fernando specifically will hear it, because he is the one who would have to do the firing.

> "Staff for the manual workflow at opening. That is the only thing either of us can verify on day one, and I am not going to tell you to hire thin against a date of mine. What I will give you in Phase 1 is the sequence: which roles

> change as each piece goes in, and in what order, so each one is a decision you make later rather than a commitment you make now. Planned, attrition and role changes absorb most of it. Discovered, it becomes a layoff."

FRAMING, NON-NEGOTIABLE. Never say automation **reduces headcount.** Always say automation **changes what your people do.** For a greenfield center scaling from zero that is also the *truer* claim: no existing job is eliminated, they are avoiding hires otherwise needed around month eighteen. Growth absorption, not reduction. Same economics, no layoff implication, and accurate.

A staffing plan tied to the automation roadmap is a Phase 1 deliverable: hire for the manual workflow at opening, then the sequence in which roles change as each piece goes in, each piece an explicit later decision. This is program-management work no vendor will ever produce. **It stays VERBAL for the 10-15 Luis session and is NOT on slide 29.** Decide with Luis whether it becomes a named slide deliverable before Fernando. Luis knows how Fernando thinks about payroll; Rick does not.

"Didn't a bunch of AI healthcare companies already die doing this?"

"Yes. Olive AI raised an enormous amount of money, shut down in 2023, and sold its prior authorization assets to Waystar and Humata Health. It sold broad automation to hospital systems on long implementation cycles. That is close to the opposite of one person building a narrow agent for one site he can watch directly."

"Why should we not just buy a prior auth vendor?"

"The category exists: Availity, Waystar, Cohere Health, Humata Health, Infinitus. It is priced and implemented for health systems and large groups, and a lot of it is payer-funded. At your size, whether that is one center or three, you are the customer they take last and configure least. And the appeal gap in the KFF data is not a technology gap. Nobody has time to file the appeal. That is a workflow problem that a narrow agent and a human sign-off step fix, and a vendor built for a hospital system does not."

"This rule doesn't cover most of my commercial book." ⚠️ the one that breaks the regulatory claim if you let it

He is right, and you say so first. CMS-0057-F binds Medicare Advantage, Medicaid, CHIP and exchange plans. It does not reach employer-sponsored commercial plans; CMS was asked directly about the people in employer coverage and declined for lack of authority. In El Paso County, employer-based-only coverage is 268,142 people, 31.8% of the insured universe, seven times the two military channels combined (38,832, 4.6%). So the rule does not automate prior auth for the largest block of the commercial book, which is where the margin is.

Say: "Correct. The rule reaches Medicare Advantage, Medicaid, CHIP and the exchange plans. It does not reach your employer-sponsored commercial plans, and that is the largest single block of coverage in this county. I am not selling you a compliance deadline. I am pointing out that the payers who move first set the standard the rest follow, and your build window lands exactly on that transition. The burden is real on every payer, the margin is in commercial, and the process fix pays for itself whether or not a given plan is on the API."

The demotion is deliberate. CMS-0057-F is the reason the window is now, not the reason to do this. For commercial plans outside the rule, the design falls back to EDI 278 where supported, and otherwise a complete packet prepared for a human to submit. That was always the design; this objection just makes you say it earlier.

"Most of our volume will be Fort Bliss and the VA." Or: "What about TRICARE?"

Source for every number here: ACS 2024 5-year, table B27010, El Paso County. Coverage of residents, not imaging volume.

Say: "I looked that up, because you would know better than I would. In El Paso County, TRICARE-only and VA-only coverage together are under five percent of the insured population, about thirty-nine thousand people out of eight hundred forty-four thousand. Commercial and Medicare are where the volume and the margin are. The military channels matter for front-office workflow, not for the business case."

The exact figures if he wants them: TRICARE-or-military-only 33,795 (4.0%), VA-only 5,037 (0.6%), combined 4.6% of the 844,384 insured universe. Employer-based plus direct-purchase 320,020 (37.9%). Do not convert any of these to a share of his studies; hand him that: "Volume will skew older than coverage does. You would know the conversion better than I would."

The non-obvious number that matters more to a greenfield pro forma than either military channel: El Paso County's uninsured rate is 21.6%, about 183,000 people. Self-pay policy and point-of-service collection are the real operational factor, and they are a front-desk scripting and workflow problem that the intake design has to own.

If he goes plan by plan on TRICARE: "El Paso is TRICARE West, and TriWest has been the West Region contractor since January 1, 2025. For TRICARE Select and TRICARE For Life, MRI, CT and PET need no referral and no pre-authorization. For Prime, non-active-duty, specialty care and some diagnostic services need a PCM referral, and the regional contractor issues the referral and pre-authorization together. For active duty on Prime, it is a referral for anything the PCM does not provide, plus pre-authorization for all specialty care. If a Prime enrollee self-refers, they are on the point-of-service option and their out-of-pocket goes up. That becomes a front-desk conversation, which is a staffing and scripting problem, not a payer problem."

If he goes to VA: "VA community care is referral-and-authorization driven, not an approve-deny gate like commercial prior auth. Texas is Community Care Network Region 4 and TriWest is the administrator. VA pays community providers at the Medicare rate unless there is a contract rate; that is 38 CFR 17.4035. VA reimburses only for services explicitly authorized in the referral, so anything outside the SEOC scope needs a Request for Service approved before you deliver the care, and VA processes one in three business days. As of September 8, 2026, VA no longer requires precertification and the precertification portal is offline. The referral and the SEOC are what govern now." Never say "VA requires precertification." Never give a referral expiry in days; the regulation caps an episode of care at one calendar year.

Neither channel is reachable by the FHIR API story, and you say that before he does: "TRICARE is not mentioned once in the final rule. On VA and DoD, CMS wrote that these are independent agencies over which HHS has no authority. Those two stay referral-and-SEOC workflow, which is intake and tracking discipline, not an API."

"We are opening three centers." Or: "Does any of this change at three?"

"Three makes the design work more valuable, not less. The specification gets written once and deployed three times, and the four contract terms get negotiated once across all three. Three centers is a reason to buy the specification, not a reason not to." Do not assume a count. Use the number he gave you at the open; if he did not give one, ask. The price line does not change.

"Is this one system for all three centers?" Or: "Why not one shared setup?"

Slide 20. "One instance per center, not one shared stack. That costs more, not less: three hosts, three databases, three sets of credentials, three audit logs, and the recurring infrastructure line sits in each center's budget rather than once. I am recommending it anyway, and the reason is governance, not technology. Separate principals, separate books, and a line a regulator, an auditor, or your lawyer can see without reading code. Share a back office if you want to; that is staff and logins, with every session scoped to one center. The specification is written once either way. The data stays apart."

Do not state as fact that the three centers share anything. Your 10-10 email to Luis assumed one shared back office and one shared vendor stack; on 10-14 you learned the centers have different principals. Settle it with Luis on the 15th. Either answer fits the slide; what changes is how many vendor contracts carry the four access terms, and whether Phase 2 writes one set of mappings or three. If he asks whether reporting can span the three: "Later, as an explicit decision by all the principals, as a read-only view over three databases. Never a merged one."

"Can RadAssist write our reports?" Or: "Is that the records piece?"

Slide 20, and be exact, because Luis is at the table. "RadAssist today is calcium scoring, thyroid, and the spine module you just watched. For calcium, Luis snips the scanner's report, pastes it, it reads the numbers and drafts the report from his template; for thyroid and spine he enters what he graded and the impression writes itself in his order. He confirms every value before anything applies. The agent drafts, the radiologist signs. At your center the path has a name: DICOM worklist and query for the study, HL7 version 2 ORU to carry the signed report into the RIS. Both are contract terms from slide 3, and I will verify the inbound result path against the RIS you pick before you sign, because not every RIS documents one."

Then the distinction, unprompted: "Requesting medical records is the other direction, priors and outside records coming in, and it is a different build. DICOM query and retrieve where a facility grants it, a request out and DICOM import where it does not, and from January the payer's Provider Access API for the claims record, in-network only, documents and not images. Which one did your partner mean?" Write the answer down. It changes Phase 2.

Never say RadAssist reads DICOM or touches the PACS; it does neither today. Never give a date for the extension. If the FDA question lands here: the radiologist's own numbers, the radiologist's own template, the radiologist's signature; it formats what the scanner measured and does not interpret an image.

"What if it makes something up?"

"A person signs every payer submission and every appeal, and the system never recommends a study or judges whether one is appropriate. That signature is deliberate design, and it is also where the FDA line is." If he asks whether the sign-off relaxes later: "Only against a written criterion. Sign-off moves from every item to a sample after a defined volume at a defined accuracy, measured, and only with your approval. Until then, every item."

"Could you just build our RIS?"

"No, and I will tell you why. Your RIS is a system of record. If it stops, your center stops. That needs a vendor with a support organization and an uptime history, which is not one person. My value to you on the RIS is on your side of the table: picking it and negotiating the terms so you own your own data. I would rather be the person who makes sure you can get your data out of it than the person who holds it."

Do not hint at a RadAssist RIS tonight. It is logged for after this engagement in BACKLOG-POST-ENGAGEMENT-2026-10-06.md. Raising it at this dinner puts you on both sides of the slide 3 contract advice.

"What about electronic health records?"

"I am not building one. An imaging center's record environment is the RIS plus the PACS. The referring physicians' EHRs are outside systems: you receive orders from them and send results back, over HL7 version 2 order and result messages, ORM and ORU in practice, plus DICOM for the images. That is the whole integration surface, and it is what the four contract terms protect."

"Could you run a local model for this?" (only if asked; do not volunteer)

"Yes, and there is a specific reason it matters. My model vendor's patient-data agreement does not cover the browser-driving tools, which are exactly what you need for a payer with no API. A model running inside your own

environment does not have that problem, because the data never leaves your building. It also makes the Texas storage requirement trivial. I would scope it in the specification rather than promise it tonight, because portal automation is the most fragile thing on this list."

Where it fits: the no-API payer portal (the single biggest unsolved piece, and the most brittle), Texas SB 1188 residency (trivially US-resident), and high-volume, low-complexity classification such as document sorting and order parsing. What it does not replace: the reasoning-heavy work. Drafting an appeal with clinical rationale wants the strongest available model. The architecture is a model-agnostic service layer with routing by data sensitivity and task difficulty, which is the same tool-first principle as everything else in the design. **Do not promise portal automation at the dinner.**

"Can your AI vendor even handle patient data?"

"Anthropic's business associate agreement covers the parts of the architecture I would use: Skills, Web Search, Memory, the Bash and Text Editor tools. It does not cover Computer Use or Web Fetch, which are exactly what you would reach for to drive a payer portal that has no API. So the prior auth design goes through APIs, EDI, or a human step. I will not promise screen-driving a portal with patient data, because I cannot do it inside the agreement today."

Add: "The coverage depends on which version of the agreement is signed and when. I will confirm the exact envelope in writing before anything with patient data is built."

"Won't regulators come for this?"

FDA: slide 21. "The statute excludes administrative support software from the device definition and names appointment schedules, claims and billing, business analytics, and determination of health benefit eligibility. Every candidate on the scorecard is on that list. The agents stop at the clinical boundary: nothing interprets an image, selects a protocol, or recommends a study."

Texas: "SB 1188 requires AI disclosure for diagnostic use. None of this is diagnostic. It also requires US-only storage of records with Texas patient data, and I will verify every component for that before promising anything."

California, if it comes up: "The state that has legislated hardest on AI patient communication, California, expressly excludes appointment scheduling and billing from its disclosure law."

"What exactly is an AI agent?" (slide 6)

"Two kinds. One does a job inside your systems, like a very consistent employee who works one desk and asks a person before anything goes out. The other you text or call, and it tells you what happened in your business, plus a report every Monday." Then the four guardrails: a person signs anything clinical or outbound; numbers are calculated and checked, never guessed; HIPAA BAAs; data stays in the US.

"When can I see it working?" (slide 11)

"In the first phase, you will click through a working prior authorization demo on synthetic data, no real patients." Open the demo, https://rfinlay.github.io/imaging-priorauth-demo/ on the laptop. Never show a link you have not opened that day.

"Exa is building something like this." (slide 11)

Never disparage Exa. "They may well, and if theirs is good, use it. What I build is vendor-neutral: it runs on your data and your contract terms and works with whatever RIS and PACS you pick, Exa included. The four access terms protect you either way."

"Is this going to be a product you sell to other centers?"

Slide 24. "Not off the back of your build. I keep the rights to my own product, and you hold it under a license you can never lose. What I will not do is make you one logo on a page. This center is the reference implementation, you are the priority, and your service terms are fixed in writing no matter what I do next. And I am not going to promise you the version of this where it gets sold to everyone else, because you already know what that does to the person answering your phone."

RadAssist licensing stays out of this room. Luis sees resale potential at about $2,500 one-time per center; that is a private conversation with Luis, never in front of Fernando.

A flat "no" would be false under the slide 23 ownership structure, and he would see that in the redline. The commitment is about his service level, not about your business model.

Say it without the words. The earlier draft of this answer quoted "and then we scale it" back at him. The point survives without the phrase, and the phrase is on the never-say strip for a reason: it is the exact sentence he heard before the company he built stopped being his to run. Name the consequence, never the slogan.

"Our last vendor promised the moon."

"I know. That is why slide 24 exists. I am telling you to buy two of the things you asked about from someone else. The things I am proposing to build are the ones where the evidence is strongest and the vendors are built for someone much larger than you."

Questions you should ASK them

These are worth more than any benchmark you could quote. Ask them early, ideally during the open, and write the answers down.

1. "**Luis told me you built and sold imaging here in El Paso before this, but I would rather hear it from you than secondhand. What did you build, and how many centers were you running at the end?" ASK THIS FIRST, before asserting anything about his history.** It lets him name his own companies and numbers, and it is the safety valve on every fact in this document. Then: "**What did front desk and prior authorization actually cost you, as a share of operating expense?**" No defensible published benchmark exists, so his P&L is the only number that matters.
2. "**What would you do differently this time, knowing how the last one ended up?**" Open question, asked at the close. **Southwest X-Ray is CONFIRMED as his** (Rick confirmed 2026-10-01), so do not ask whether it is; asking a question he already answered reads as not listening. It now runs five El Paso locations as an Akumin company, and **Akumin went through Chapter 11 in 2023**. So he is opening in direct competition with a national roll-up that owns what he built, and the buyer had financial trouble afterward. **Do not raise the bankruptcy. Do not say "scale" or "platform" anywhere near him.** If he raises any of it, stop talking and listen; his answer tells you what he actually wants from this center, and it is probably control.
3. "**Which RIS and billing vendors are you looking at, and have you signed anything yet?**" This tells you whether the four contract terms on slide 3 are still free.
4. "**How many prior authorizations a day were you running at steady state, and who did them?**" Gets you his real volume and whether he already lived the 40% statistic.
5. "**You came up as an RPA, so you have been in the room with the patient. When you think about automating the front end, where is the line for you, what has to stay human?**" Almost no software vendor knows what an

RPA is. This respects his clinical origin instead of treating him as the business guy, and his answer is your scope boundary from the person whose opinion decides the sale.

6. "**We are two people. If you were in my chair pitching you, what would you want to see about continuity and about getting out, before you would put us anywhere near your schedule?**" Hand him the objection and let him specify his own terms. Whatever he names is your path to a yes.
7. "**What was your cancellation rate, and what did you do with the empty slot?**" Tests whether the 22.3% framing matches his experience before you lean on it.
8. "**What is your payer mix going to look like?**" The CMS-0057-F tailwind covers Medicare Advantage, Medicaid and CHIP, and exchange plans, not employer-sponsored plans, TRICARE, or VA. If he expects heavy commercial, say so and adjust the weight you put on the 2027-01-01 date. County coverage shares are now verified (ACS 2024 5-year, B27010): employer-based-only about 268,000, direct-purchase-only about 52,000, Medicaid-only about 134,000, Medicare-only about 48,000, TRICARE-or-military-only about 34,000, VA-only about 5,000, and 21.6% uninsured. Those are coverage shares of residents, never a share of his studies; hand him the conversion. **Diligence item to name out loud: "MA penetration is the number that decides how much of your Medicare book the new rule touches. I'd pull that before we price Phase 2."** Medicare Advantage is covered by the rule, so MA penetration in El Paso determines how much Medicare volume the rule actually reaches. The research could not establish it. Do not guess a number.
9. "**Who takes the phone call when a patient describes chest pain?**" This is how you open the voice conversation without a slide. It makes the "buy, do not build" recommendation obvious before you state it.
10. "**What do you want to see on one page every Monday morning?**" This is the owner-reporting candidate. Whatever he says is the specification.

Things to say out loud that are not numbers

- "Whatever you decide about me, put those four terms in the contracts. I will give you the language."
- "I would rather tell you to buy something than build something that is not mine to build."
- "Texas has its own records deadline that is shorter than the federal thirty days. I will not quote it from memory. It goes in the specification after I verify it against the statute."
- "A reminder plus a backfill offer plus a results notice can collide with the three-per-week cap. The system needs a per-patient rate governor, not just a send queue."
- "The design engagement exists so that the bus question has a boring answer."
- "The agent drafts. The radiologist signs. Nothing clinical leaves this building without a human signature."
- "One instance per center. It costs more, and the reason is governance, not technology."

Things not to say

- Anything from the "must NOT say" list above.
- Vendor adjectives, "scale," "platform," "disrupt." He has heard every one of them.
- Any guess about Fernando's history that Luis has not confirmed.
- Any date for the build outside the slide 26 assumptions. Targets are conditional, always.
- Any building, property, address, or prior business at the site. "Your new center," nothing more.
- RadAssist licensing to other centers, in front of Fernando.

- Anything negative about Exa.
- "VA requires precertification." It does not, as of 2026-09-08. "VA dropped precertification in September 2026; the referral and SEOC govern."
- The opening date as a fact. It is the center's target. Say "your target" or "projected," never "you open in June."
- That the three centers share a back office, a vendor stack, or anything else, as a fact. It is unresolved; ask Luis.
- Any principal's name you have not heard Luis spell. Fernando Escarzaga is confirmed; nobody else is.
- That RadAssist reads DICOM, touches the PACS, or reads an image. Calcium reads a screen capture the radiologist pastes; thyroid and spine take what he graded. Image auto-fill is "a future item with a regulatory path," never a promise.
- The word "scale," the word "platform," or Akumin, anywhere near the RadAssist demo. For Fernando it is control and simplicity: his template, his rules, his signature, plain text into whatever system he buys.